



AUTISM SPECTRUM DISORDER

Broad Topics Seminar 9/10/26

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Discussion Questions:

- What have been your experiences with assessing for and treating ASD?

ASD

Criteria A:

Persistent deficits in social communication and social interaction across multiple contexts, as manifested by the following:

- Deficits in social-emotional reciprocity (e.g., limited social overture/responsiveness, poor reciprocity, difficulty sharing/expressing emotions)
- Deficits in nonverbal communication used for social interaction (e.g., eye contact, directed facial expressions, gestures)
- Deficits in developing, maintaining, and understanding relationships (e.g., social motivation, limited understanding of role, difficulty understanding relationships, limited play).

ASD

Criteria B:

Restricted, repetitive patterns of behavior, interests, or activities, as manifested by at least two of the following:

- stereotyped or repetitive motor movements, use of objects, or speech (e.g., simple motor stereotypies, lining up toys, repetitive speech, echolalia, idiosyncratic phrases).
- Insistence on sameness, inflexible adherence to routines, or ritualized patterns of verbal or nonverbal behavior (e.g., extreme distress at small changes, difficulties with transitions, rigid thinking patterns, greeting rituals, need to take same route or eat same food every day).
- Highly restricted, fixated interests that are abnormal in intensity or focus
- Hyper- or hyporeactivity to sensory input or unusual interest in sensory aspects of the environment (e.g., apparent indifference to pain/temperature, adverse response to specific sounds or textures, excessive smelling or touching of objects, visual fascination with lights or movement).

ASD

Criteria C

- Symptoms must be present in early childhood (although may not fully manifest until older)
 - Very important for differential diagnoses considerations (e.g., anxiety, trauma, depression, etc.)

Severity: Level system (Level 1, Level 2 or Level 3)

- Based on degree of support required

Specify if:

- With or without accompanying intellectual impairment
- With or without accompanying language impairment
- Due to known genetic/ medical condition

Assessing for ASD

- The ADOS-2
 - The “gold-standard”
 - 5 different modules based on amount of language/age
 - Many services expect to see an ADOS-2 score for qualifications
- Quality clinical interview
- Self-report measures from multiple informants
- Cognitive test (high comorbidity with other ND disorders)

- Who can diagnose ASD?
 - ASD insurance waiver & IBHS

The ADOS-2

- Toddler (12 Months to 30 months)
- Module 1 (31 months/Preverbal or single words)
 1. Free Play
 2. Response to name
 3. Response to Joint attention
 4. Bubble Play
 5. Anticipation of routine with objects
 6. Response to social smile
 7. Anticipation of social routine
 8. Functional and symbolic play
 9. Birthday Party
 10. Snack
- Module 2 (Phrase speech)
 - Combines 1 and 3

The ADOS-2

- Module 3 (Fluent language, children and younger teenagers)

1. Construction task
2. Make-believe play
3. Joint interactive play
4. Demonstration task
5. Description of a picture
6. Telling a story from a book
7. Cartoon
8. Conversation
9. Emotions
10. Social difficulties and annoyances
11. Break
12. Friends, Relationships, and Marriage
13. Loneliness,
14. Creating story

The ADOS-2

- Module 4 (Fluent language, older teenagers and adults)

1. Construction task
2. Demonstration task
3. Description of a picture
4. Current work/school
5. Daily living
6. Plans and Hopes
7. Telling a story from a book
8. Cartoon
9. Conversation
10. Emotions
11. Social difficulties and annoyances
12. Break
13. Friends, Relationships, and Marriage
14. Loneliness
15. Creating story

Assessing for ASD:

ADOS-2 Module 1 Demo:

- https://www.wellchildlens.com/video_room/detail/process-and-diagnosis/psychologist/D49A200BF77C%20C48324F1%201479%206170%20DDD1

Discussion Questions:

- Any questions or reactions from video clip?
- While considered the “gold standard,” what may be some limitations of the ADOS-2?

Early Signs

- Babies later diagnosed with ASD may demonstrate reduced:
 - Vocalizations, movement, faces (i.e., more interested in nonsocial objects)
 - Attempts to seek physical contact
 - Orientation towards faces, movement
 - Social smiles
 - Responsiveness to name*
- By one year, may demonstrate reduced:
 - expressive and receptive language (EI and speech)
 - Use of nonverbals (including eye contact)
 - Environmental exploration

Early Signs

- Preschoolers may demonstrate reduced:
 - social chat (vs requests)
 - Pointing/showing objects for sake of social engagement
 - Gestures, directed facial expressions
 - Empathy (comforting others who are hurt)
 - Full range of facial expression
 - Responsiveness to name
 - Interest in peers
 - Ability to follow along in social games
 - Imaginative play
 - Ability to attend to social information (social stories)
- Developmental Regression occurs in 30% of cases

Theory of Mind



Joint and Social Attention

When two people purposefully pay attention to the same object with social intent:

- Important for communication, sharing experiences with others, and learning

- Typically emerged around 9 months

 - Incorporate other nonverbals around 12 months

- Children with ASD initiate less initiation and responsiveness to JA

 - Plays a role in social difficulties

Facial and Emotional Perception

- Individuals with ASD demonstrate difficulty in face processing abilities
 - Facial recognition difficulties evident in middle childhood
- Milestones of typically developing children:
 - 6-months typical infants respond differently to emotions in environment (smile more at happy faces)
 - 9-12 months use social referencing of others' emotions
 - 12 months beginnings of ability to respond well to others' distress
- Children with ASD show impairments in ability to recognize affective cues, failure to attend and recognize observable beginning at age 2
- Despite this, can still distinguish between many emotions

Imitation and Play

- Children with ASD show difficulties imitating others' body movements/simple actions
 - May see reduced interest in copying caregiver in preschool age (e.g., cooking, cleaning, gardening, etc.)
- Absence of / limited pretend play in early childhood
 - May engage in pretend play as language increases or with scaffolding
 - May still prefer mechanical, cause-and-effect, repetitive play (non-functional play)

Language Concerns

- Often cited as initial concern
- About ¼ of children with ASD will not communicate verbally despite intervention (this estimate may be changing with improvements in assessment)
- Language patterns:
 - Echolalia (delayed or immediate verbatim repetition of heard words or phrases)
 - Atypical prosody/intonation/volume (abnormal rhythm, stress, intonation, and loudness)
 - Use of neologism or idiosyncratic language (made up words)
 - Pronoun errors (use of “you” instead of “I” when referring to self)
 - Scripting (can vary in how “integrated this feels)
 - Repetitive speech

Language Concerns

Despite these, most impairments include pragmatic / social use of language

- providing unnecessary, irrelevant, or excessive details in conversation
- Perseveration of topics
- Lack of to-and-fro
- Pedantic speaking (overly formal for situation) on a particular topic
- Inappropriate shift to new topic without context or consideration for the listener
- Lack of responsiveness to other person in conversation (not responding to topic introduction)
- Fewer spontaneous offering of information
- Reduced response to questions
- Comprehension challenges
 - These differences are even present in folks with intact IQ and verbal skills

Language Concerns: Nonverbal Differences

Difficulties ranging from integrating nonverbals alongside verbal communication, limited use of nonverbals, and/or challenges interpreting nonverbal social information

- Emerging in early childhood:
 - Limited use of gestures
 - Descriptive: to describe something
 - Arms out to show something is this big
 - Instrumental: to get something
 - Finger wag for come here
 - Hand up for stop
 - Informational: to communicate
 - Head nod, shrug, sign language
 - Conventional:
 - Waving, clapping
 - Emphatic/Emotional
 - Limited eye contact
 - Limited directed facial expressions

Other challenges

- Cognitive and behavioral rigidity may manifest as
 - Intense and restricted interests
 - Ritualistic behaviors
 - A high degree of insistence on sameness (e.g., taking the same route home from school, using the same cup at every meal, only eating the same brands of certain foods).
 - Emotional outbursts
 - Difficulty “switching gears” and adapting

Biopsych Considerations

- Social Motivation Theory of ASD
 - Children with ASD often have differences in serotonin and dopamine
 - Reward circuitry in brain is less responsive to social stimuli
- Theory of reduced oxytocin
- Brain areas studied: amygdala , medial prefrontal cortex, temporoparietal junction, superior temporal sulcus, fusiform gyrus, basal ganglia (caudate, putamen) anterior cingulate cortex
 - These regions coordinate:
 - Social salience, face processing, perspective taking, social reward
 - Habit formation, cognitive flexibility, behavioral inhibition
 - May relate to challenges with predictability and need for sameness (social situations are also highly unpredictable)

Cultural Considerations

- Children from low-SES families often diagnosed later or missed
- Girls are often missed due to criteria haven been developed with a focus on how boys present
 - Girls are *typically* more socially-savvy and socially motivated
 - Socialized to be empathetic and caring
 - Girls often begin to “mask” at an earlier age
- Movement from deficits based understanding of ASD
 - Strong identity and community
 - Person first vs identity first language

Discussion Questions:

- Pros and Cons of increased awareness of neurodiversity in general public
- Is ASD “trendy?” How can we navigate misinformation with patients?
- Other cultural considerations regarding the assessment and treatment of ASD?

Interventions

- IBHS
 - Social skills group
 - Intensive-OP/family based /“wrap-around”
 - Day treatment programs
 - ABA
- Individual/Family therapy
 - PCIT/PMT
- Medication?

Discussion:

- ABA and its controversy – what are your thoughts, opinions, experiences?